

AI-Powered HMS Synthetic Clinical Dataset

Encounter Note

Patient Name	Ta Thi Thao	MRN	MRN-0100
DOB	1995-07-02	Gender	Female
Encounter Date	2024-03-28	Department	Obstetrics & Gynecology
Attending Provider	RN Jamie Owens	Status	discharged

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Ta Thi Thao is a 30-year-old patient in Obstetrics & Gynecology presenting for follow-up of routine follow-up with screening and safety counseling. PMH includes routine follow-up, denies heavy bleeding, pain, or fever unless noted. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include prenatal vitamin daily; ferrous sulfate 325mg daily; acetaminophen 500mg QID PRN. Allergies: NKDA.

Objective

Vitals: BP 140/65 mmHg, HR 82 bpm, RR 18/min, Temp 36.6 C, SpO2 94% RA, weight 71 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-04-30	Creatinine	1.16	mg/dL	0.7-1.3	Normal
2026-04-30	eGFR	73.0	mL/min/1.73m2	>=60	Normal
2026-04-30	Glucose	110.0	mg/dL	70-99	High
2026-04-30	HbA1c	5.4	%	<5.7	Normal
2026-04-30	BNP	78.0	pg/mL	<100	Normal
2026-04-30	AST	33.0	U/L	10-40	Normal
2026-04-30	ALT	31.0	U/L	7-56	Normal
2026-04-30	Potassium	4.2	mmol/L	3.5-5.0	Normal
2026-04-30	Hemoglobin	11.9	g/dL	12.0-17.5	Low

Assessment

Primary assessment: prenatal or gynecologic follow-up with anemia and glucose screening needs. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Review CBC, glucose screen, and BP trend. Educate on warning signs including severe headache, bleeding, or decreased fetal movement. Schedule follow-up per OB protocol. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. prenatal or gynecologic follow-up with anemia and glucose screening needs. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Obstetrics & Gynecology.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: encounter_note.